VANDERBILT HEALTH | Specialty Pharmacy

Impact of Expanding Belzutifan to an Integrated Health-System Specialty Pharmacy

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BACKGROUND

- Belzutifan is a first-in-class medication, indicated for von-Hippel Lindow (VHL) disease associated renal cell carcinoma (RCC), pancreatic neuroendocrine tumors, and central nervous system tumors
- One year after approval, Vanderbilt Specialty Pharmacy (VSP) gained access to the limited distribution network (LDN) to dispense belzutifan

METHODS

Objective

To determine the impact of expanding belzutifan access to VSP on time to first fill from treatment decision, number of gaps in therapy after initiation, and adherence

Study Design
Single-center, retrospective, cohort study

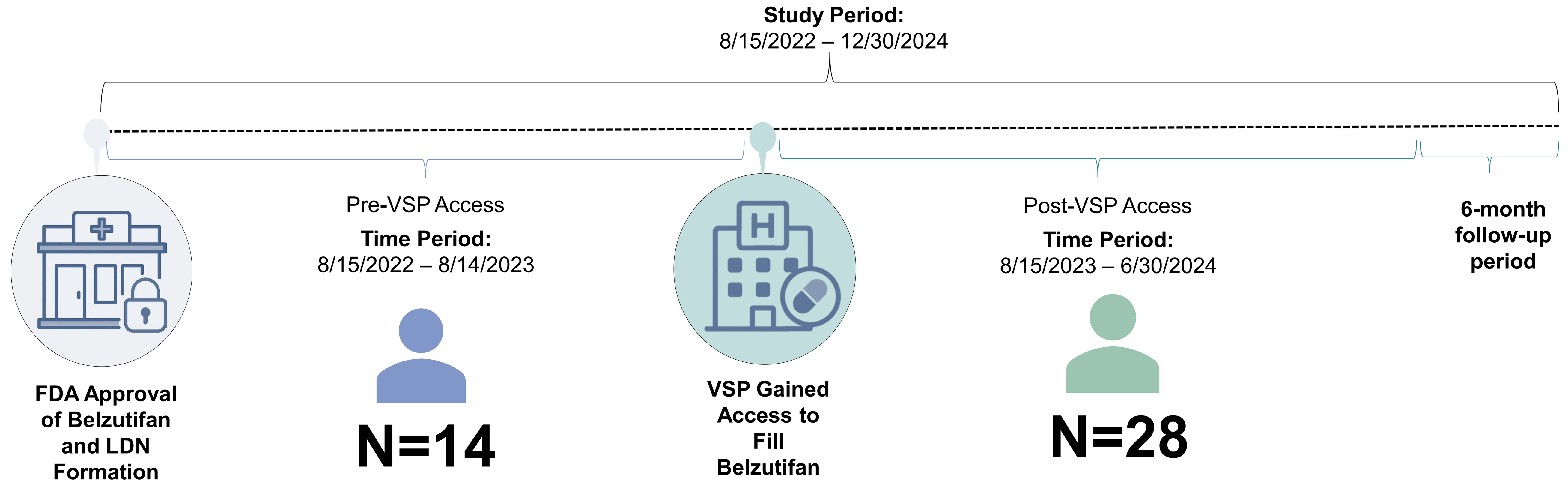
Study Setting
Vanderbilt Ingram Cancer Center, Integrated HSSP

Study Sample
Patients starting belzutifan for RCC or VHL between 8/15/2022 and 6/30/2024

Outcomes

1. Time to first fill
2. Treatment gaps (>1 day without treatment) over 6 months
3. **Adherence** over 6 months using proportion of days covered (PDC) in patients with at least 3 fills

Figure 1. Study Timeline and Attrition



After VSP gained access to fill belzutifan, 12/14 patients (86%) converted to filling with VSP. For this analysis, these patients were included in the pre-VSP access group.

Abbreviations: VSP = Vanderbilt Specialty Pharmacy; HSSP = health-system specialty pharmacy; PDC = proportion of days covered; IQR = interquartile range

HIGHLIGHTS

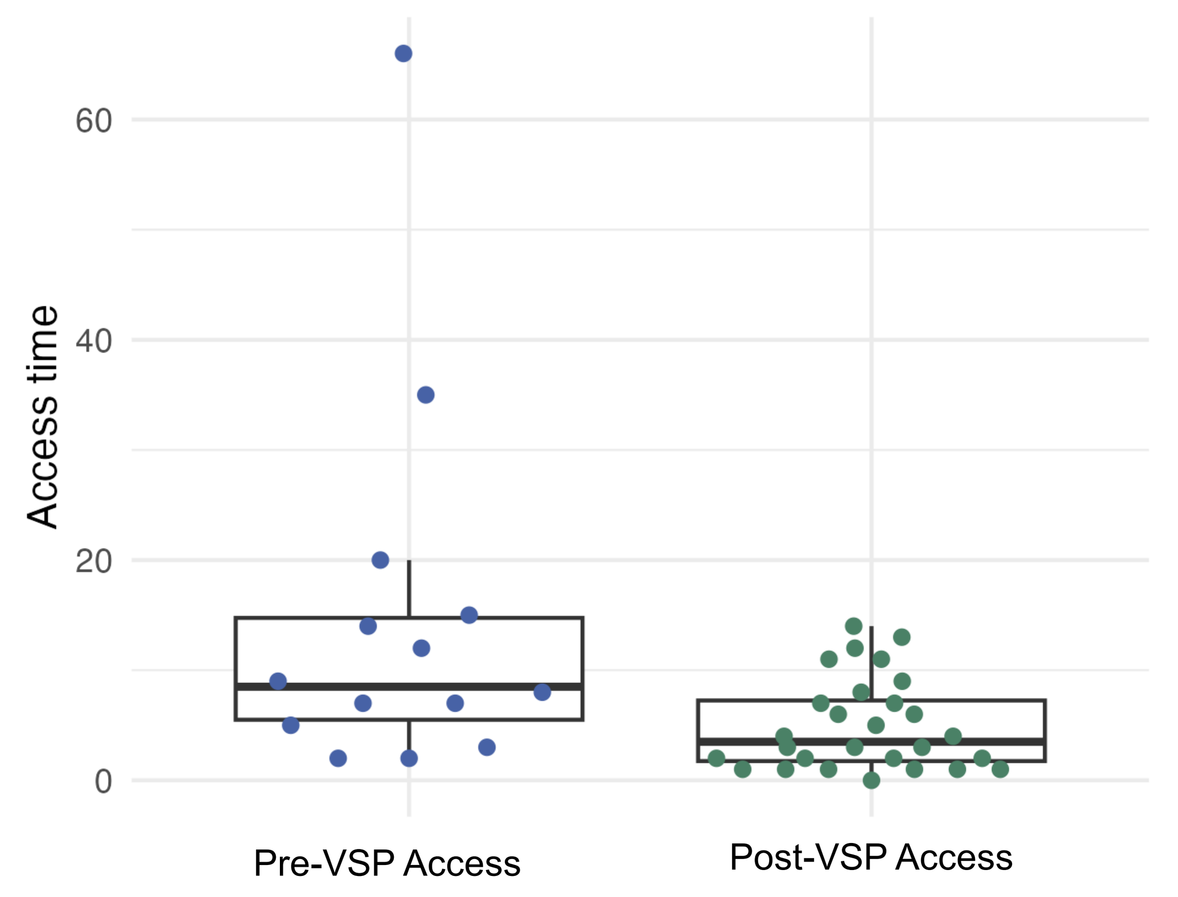
- Patients who filled a LDN medication once VSP gained access initiated treatment more quickly, had fewer treatment gaps, and improved adherence compared to those who began treatment before VSP gained access.
- These findings support prior studies showing HSSPs outperform non-HSSPs when managing patients on LDN medications.

RESULTS

Table 1. Patient Demographics (n=42)

Characteristic	Pre-access n=14	Post-access n=28
Sex: Female, n (%)	9 (64)	10 (36)
Age (as of June 2024), Median (IQR)	41 (34 – 52)	67 (58 – 70)
Race, n (%)		
Black or African American	2 (14)	1 (4)
White	10 (71)	25 (90)
Other	2 (14)	2 (7)
Pharmacy insurance, n (%)		
Commercial	8 (57)	14 (50)
Medicare	6 (43)	14 (50)
Indication, n (%)		
Renal cell carcinoma	0 (0)	22 (79)
Von Hippel-Lindau disease	14 (100)	6 (21)

Figure 2. Time to Initiation of Therapy (First Fill)



Median time to belzutifan first fill **decreased 5 days** following VSP gaining access (p=0.008)

Pre-VSP Access: 9 days [IQR 6-15]

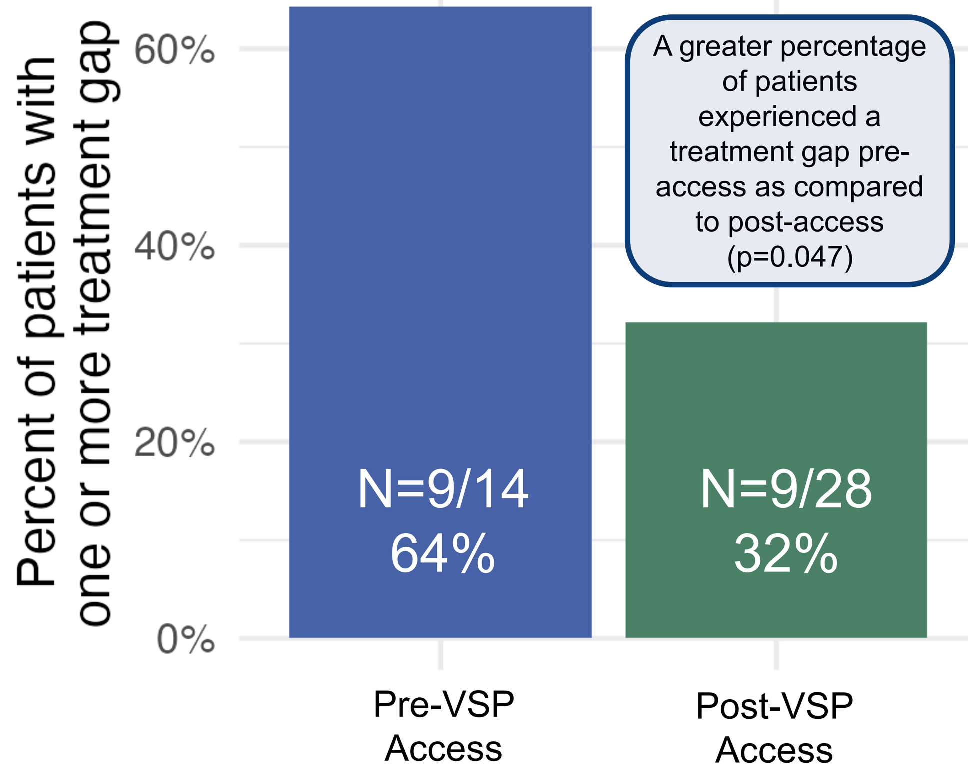
Post-VSP Access: 4 days [IQR 2-7]

Figure 3. Gaps in Therapy

Reasons for gaps in therapy

- *Patient adherence to follow-up* (Pre-VSP = 3 vs Post-VSP = 0)
- *Insurance delay* (Pre-VSP = 2 vs Post-VSP = 7)
- *Coordination of care* (Pre-VSP = 0 vs Post-VSP = 1)
- *Financial assistance needed* (Pre-VSP = 3 vs Post-VSP = 1)
- *Health* (Pre-VSP = 3 vs Post-VSP = 1)

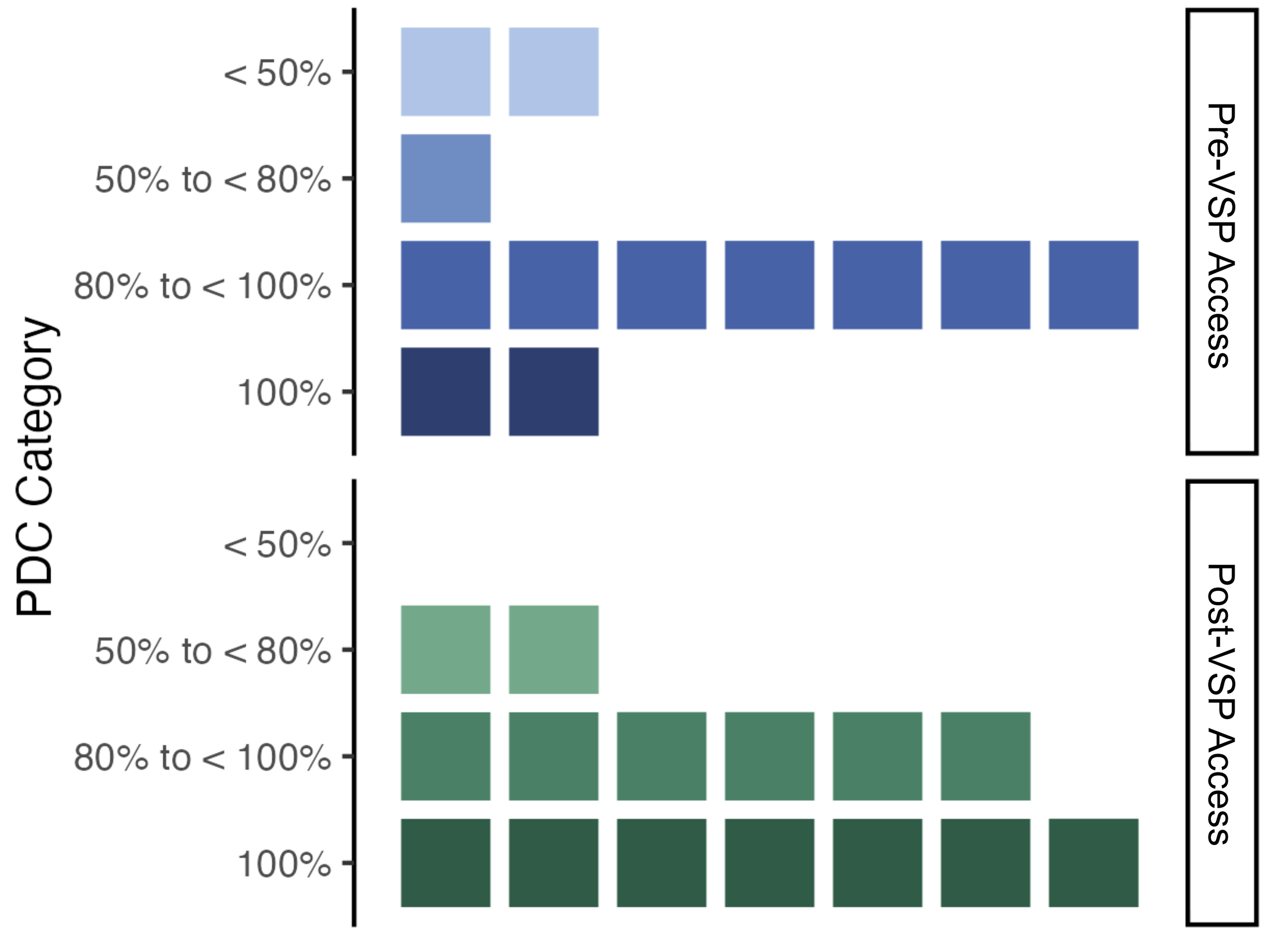
Post-VSP therapy gaps were more commonly **due to non-modifiable reasons,** such as insurance delay, patient adherence to follow-up, or financial assistance needed



Patients could have experienced multiple reasons for the treatment gap; therefore, the number of gaps might not equal the number of reasons for the gap

Figure 4. Adherence

Patients who filled belzutifan prior to VSP obtaining access had a median PDC of 91% (IQR 73% - 98%, n=12) compared to a median PDC of 98% (IQR 86% - 100%, n=15) post-access (p=0.129)



Each box represents the PDC of one patient during the 6-month follow-up period